LIBBY — PATIENT/CAREGIVER TRACK

pdac-recurrent-cldn18-2-kras-g12r-tp53-y220c-p25m

Plain-language summary for patient and caregiver

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This is decision-support information, not a treatment plan. Please share it with your oncologist before making any decisions based on what's here.

What this page is

Libby searched published studies and open clinical trials for treatments aimed at the specific features of your cancer, the ones you told us about. This page walks through what came back: one first step, five treatment options worth discussing with your care team, two options you may have heard of that are on the page as not recommended, and one that fits your tumor well but cannot be reached right now. Where the research has real numbers, we give them in plain terms. Where it does not, we say so instead of dressing anything up.

One thing to be honest about at the start. A recurrence about four months after finishing chemotherapy is a serious situation, and this page does not pretend otherwise. It also does not mean there is nothing to do. There are real options here, and there is one clear next step.

What we know about your cancer

You have pancreatic ductal adenocarcinoma, the most common form of pancreatic cancer. You went through a long and hard course of treatment: six cycles of chemotherapy (mFOLFIRINOX) that shrank the tumor, then surgery in December 2025 that removed it completely with all 34 lymph nodes clean, then four more cycles of chemotherapy afterward, finished as planned. That is ten cycles of chemotherapy in total. In July 2026 a new mass appeared in the body of the pancreas on a scan, sitting close to blood vessels. It has not yet been biopsied.

Testing on the tumor removed in December found several things worth knowing:

- **CLDN18.2.** About 60% of the tumor cells carried a surface protein called claudin 18.2, at moderate-to-strong levels. Several experimental treatments are built to home in on this protein, so this is a genuine asset. It comes with a catch we explain under Option 6.
- **KRAS G12R.** A change in a gene called KRAS, of a specific type called G12R. KRAS is the engine driving most pancreatic cancers, and a new class of drugs now targets it directly.
- **TP53 Y220C.** A change in a gene called TP53. There are early-stage drugs aimed at this exact change, though, as it turns out, they tend not to work well when KRAS is also mutated. They are not among the options below for that reason.
- **MSI-negative, low mutation count.** Two markers that predict whether standard immunotherapy will help. Yours predict it will not. That closes one door, and it is addressed head-on under Option 7.

One fact shapes this whole page: every one of those results comes from the tumor removed in December 2025, not from the new mass. Ten cycles of chemotherapy happened in between, and cancers can change under that pressure. The new mass has not been sampled or tested at all yet.

A recurrence this soon after chemotherapy, despite a clean surgery, tells us the cancer did not stay sensitive to the chemotherapy you received. That is part of why the options on this page look beyond simply running the same regimen again.

What you told us matters most

You said clinical trials are welcome, including ones that require travel, and you set no geographic limit. You named no side effects you refuse to accept and ruled out no type of treatment. The options below reflect that: most of them are trials, and the list leans toward effectiveness over gentleness. One note: we assumed you would weight effectiveness more heavily than side effects, because you did not say otherwise. That was our default, not your words, and it is worth revisiting with your team.

One practical flag. After ten cycles of oxaliplatin-containing chemotherapy, most people carry some lingering numbness or tingling in the hands and feet. Yours has never been formally graded, and several trials will not enroll people whose numbness is above a certain level. Getting it graded is quick and matters more than it sounds.

The first step everyone agreed on

Before any treatment decision, the new mass needs a biopsy, along with updated scans, a fresh stain for CLDN18.2, and new genetic sequencing. This can feel like a delay when the instinct is to start treatment now. It is not stalling. It is the step that makes everything else possible.

Here is why. Every test result on file describes a tumor that no longer exists, removed from your body eight months ago, before four more cycles of chemotherapy. The biopsy answers four questions at once: whether the new mass really is the pancreatic cancer coming back, whether it has spread beyond the pancreas or stayed local, whether the KRAS and TP53 changes are still there, and how much CLDN18.2 the current tumor carries. Nearly every trial on this page requires this information before you can enroll, and one tissue sample can feed all four tests.

The biopsy itself is done with a thin needle guided by an internal ultrasound. Its risks are modest, mainly bleeding and a small chance of pancreas inflammation afterward, and the blood vessels near the mass may shape how your doctors approach it. If it turns out the mass cannot be sampled safely, a blood test that picks up tumor DNA in the bloodstream can answer some of the same questions.

One caution about reading the CLDN18.2 result when it comes: on a small needle sample, the stain misses the protein about half the time when it is really there. So a positive result settles the question, but a negative result does not prove the protein is gone, and it should not close that door by itself.

There is a clock, and it runs the opposite way from what most people expect. The leading trial for the top-ranked drug below only accepts patients within six weeks of a diagnosis of metastatic disease, meaning cancer that has spread. That window starts when the scans and biopsy make the call, not when you feel ready. So the phone call to the trial site should happen before the biopsy result comes back, not after, so that everything is teed up if the answer opens that door. And starting chemotherapy first can quietly shut it, because it changes which trial groups you fit into. If chemotherapy is on the table, ask the specific question: will this dose close any trial doors? This is probably the single most useful practical fact on this page.

Two more things ride alongside and need no tissue at all, just blood. First, a dedicated germline genetic test, explained under Option 4. It is the cheapest, highest-value item on this list, and guidelines recommend it for every person with pancreatic cancer. Second, the formal grading of your leftover numbness. Both can be ordered this week.

And the flip side: if the biopsy shows something other than recurrent pancreatic cancer carrying these features, the treatment options below come off the table. Libby's rankings only cover treatments aimed at the specific targets you told us about, so in that situation there would be nothing further ranked on this page. Standard care for pancreatic cancer would still exist and would still be worth pursuing, but that is a separate conversation that belongs to your oncologist, not to this page.

The options

Option 1 — Daraxonrasib (a pill that targets KRAS)

Only on the table if the biopsy confirms the cancer is back and still carries the KRAS change.

This is the lead option, and the best-evidenced one. Daraxonrasib is a daily pill that switches off the mutated KRAS protein driving the cancer, including your G12R type, which older KRAS drugs could not touch.

It is the only option on this page backed by a large randomized trial in pancreatic cancer. In that trial, 500 people whose pancreatic cancer had spread and grown despite chemotherapy were assigned to either daraxonrasib or more chemotherapy. Among those with KRAS changes like yours, half of the people on daraxonrasib lived longer than 13 months; among those on chemotherapy, half died within about six and a half months. The drug also held the cancer still for about twice as long. In a smaller earlier study, about one in three people saw their tumors shrink.

Now the honest part, because those numbers are not automatically your numbers. The trial studied people whose cancer had already spread and who had been treated after it spread. Your situation is different: your chemotherapy came before and after surgery, and whether the new mass has spread is exactly what the biopsy and scans will determine. Also, your specific G12R type was studied inside a larger group of KRAS changes, never on its own. The direction of the result is solid. How big the benefit would be for you specifically is genuinely uncertain, and I can't give you a precise number there without data that does not yet exist.

Side effects are real: mostly rash and gut trouble (diarrhea, nausea, mouth sores). About six in ten people on the trial had at least one severe side effect, though that was actually fewer than on the chemotherapy arm, and only about one person in a hundred had to stop the drug because of side effects. The drug is new; the first person ever to take it did so in 2022, so nobody knows what year four looks like. And because you have had pancreatic surgery, it is worth asking whether pill absorption could be an issue for you.

There are three ways to get it, and they close in different ways. The big trial has the six-week clock described above and requires that the cancer has spread. There is also an FDA-authorized expanded-access program: your oncologist requests the drug from the company, it is supplied free of charge, and you take it under a monitored protocol rather than as improvisation. That route also requires confirmed spread. A third, earlier-phase study does not hinge on that call. One sequencing note worth raising with your team: several other KRAS trials only take people who have never had a KRAS drug, so starting daraxonrasib closes those doors. That is not a reason to wait. It is a reason to plan.

Option 2 — IBI343 (an antibody that delivers chemotherapy to CLDN18.2 cells)

Only on the table if the biopsy confirms the recurrence, and the trial will re-test CLDN18.2 with its own assay.

This is recommended as the strongest of the CLDN18.2 routes you can actually reach. IBI343 is an antibody-drug conjugate: an antibody finds cells carrying CLDN18.2 and delivers a chemotherapy payload directly to them, sparing more of the rest of the body. The study behind it enrolled pancreatic cancer patients whose tumors carried CLDN18.2 in at least 60% of cells, which is exactly where your December result sits, though the trial's own lab will make the final call on fresh tissue.

The evidence is early: one study without a comparison group, reported so far only as a conference summary. In it, about one in five patients saw their tumors shrink, and most saw the cancer at least held in place for a time. Half of patients lived longer than about nine months. Here is the context that number needs: after this kind of chemotherapy stops working, switching to a different standard chemotherapy gives people a broadly similar amount of time. So this drug has not yet shown it beats the ordinary next move. It may earn that; it has not yet.

The main side effects are drops in blood counts, which about half of patients experienced at a severe level in the related stomach-cancer study. After ten cycles of chemotherapy, how much reserve your bone marrow has left is an open question your team should look at first. One more open question worth asking the sponsor: the payload is a cousin of irinotecan, a drug you have already had ten cycles of, and nobody has published whether prior irinotecan blunts it. The trial has sites in the US and Australia.

Option 3 — ASP2138 (a drug that drags immune cells onto CLDN18.2 cells)

Only on the table if the biopsy confirms the recurrence. Some parts of this trial also require that the cancer has spread rather than stayed local.

This option carries caveats. It is an early study of a "T-cell engager," a molecule with two hands: one grabs the CLDN18.2 protein on cancer cells, the other grabs your own immune cells and forces the two together. It is on the page because the door is unusually wide open, not because the evidence is strong: the trial's CLDN18.2 entry bar is low and your tumor clears it several times over, it runs at dozens of sites including many in the US, and your stored tissue from surgery may be enough to start the screening process while the biopsy is being scheduled.

The evidence side is the thinnest of the recommended-or-caveated options above this line. There are no effectiveness results in pancreatic cancer at all yet; the pancreatic patients treated so far have only contributed safety information. In stomach cancer, the drug held disease in place at twelve weeks in about four in ten patients, and that is a soft, early way to measure benefit. There is also a biological concern to weigh: this drug needs immune cells already inside the tumor to do its work, and pancreatic tumors are notoriously poor in them. That is the open question hanging over this whole drug class in this disease.

The characteristic side effect is cytokine release syndrome, an immune overreaction with fever and low blood pressure, seen in about one in four pancreatic patients in the study so far, mostly mild and managed in experienced centers.

Option 4 — Olaparib, only if a germline blood test comes back positive

This option carries caveats, and it is really about the test behind it. Olaparib is a pill approved for pancreatic cancers in people who carry an inherited change in the BRCA genes. Whether you carry one has never actually been checked. The line on your tumor report saying no incidental germline findings came from a tumor test, which is not designed to answer that question and can miss real inherited changes. A proper germline test is a simple blood draw, guidelines have recommended it for every person with pancreatic cancer since 2019, and it can be ordered this week without waiting for anything else.

Straight odds: roughly five to seven people in a hundred with pancreatic cancer carry such a change, and the fast return of your cancer after platinum chemotherapy makes a positive result somewhat less likely still. So this is a long shot. But the test is cheap, the result also matters to your blood relatives, and a positive would open an entire treatment class with an FDA label behind it, which nothing else on this page has.

If the test is positive, here is what olaparib actually offers: in its main trial, taken as maintenance after chemotherapy had the cancer under control, it roughly doubled the time before the cancer started growing again (about seven and a half months versus about four), but people did not live longer overall. What it buys is time off chemotherapy, not proven extra survival, with its own side effects (mainly anemia, fatigue, and nausea, severe in about four in ten people). Your team would also have to think through the fact that this strategy assumes another round of platinum chemotherapy works first, which your recurrence timing puts in doubt. If the test is negative, this option simply comes off the table, and nothing else on the page changes.

Option 5 — ASP5834 (an intravenous KRAS drug in its first human trial)

Only on the table if the biopsy confirms the recurrence.

This option carries caveats: it has no published results of any kind. Not on effectiveness, not on side effects. It is a brand-new pan-KRAS drug, given by vein, in its very first human study, where early participants may receive doses too low to work because finding the dose is the study's purpose.

So why is it here at all? Insurance. The two main routes to daraxonrasib require that the cancer has spread. If your scans instead show the cancer has come back only locally, those routes close on the same day, and this trial is the KRAS-targeting door that stays open: it accepts locally recurrent disease, it names your exact G12R type in its entry criteria, and its rules explicitly count a history like yours (chemotherapy around surgery, recurrence within six months) as qualifying. Being given by vein also sidesteps the pill-absorption question after your surgery. Think of it as the backup to have already scoped out on the day the staging result arrives, not as a rival to Option 1.

Option 6 — Zolbetuximab, used off-label

This option was considered but not recommended. It's on the page because Libby surfaces what was weighed and rejected, not just what was kept, and because this is a drug a US oncologist could legally prescribe tomorrow, so you may well hear about it. It is the one approved CLDN18.2 drug, and the approval is for stomach cancer.

There are two independent reasons to pass. First, the approval requires strong CLDN18.2 staining in at least 75% of tumor cells, and your December result was 60%. Second, and more decisive: the drug was tested head-to-head in a randomized trial in pancreatic cancer, added to chemotherapy, and it added nothing. People lived essentially the same length of time with it or without it. Paying for an off-label drug, with its substantial side effects, against a flat randomized result in exactly your disease is a poor trade.

One thing this should not do is discourage you about CLDN18.2 itself. Being under the cutoff for this one drug is not the same as being negative. Your 60% is above the entry bar for Options 2 and 3 and for the out-of-reach option below. The marker is still an asset; this particular drug is just the wrong way to use it.

Option 7 — Single-agent immunotherapy (pembrolizumab and similar drugs)

This option was considered but not recommended. Checkpoint immunotherapy drugs have transformed some cancers, and it is natural to ask about them. For these drugs to work on their own, a tumor generally needs one of two features: a repair defect called MSI-high, or a very large number of mutations for the immune system to recognize. Your tumor was tested for both. It has neither: it is MSI-negative, and its mutation count is very low, roughly a twelfth of the level where these drugs are approved. These are not borderline results.

Given that, single-agent immunotherapy would carry its real risks of immune side effects with essentially no realistic chance of benefit. This is not a judgment about immune approaches in general. Treatments that force an immune attack rather than waiting for one, like the engager in Option 3, remain on the table. It is specifically the checkpoint-drug-alone route that your tumor's own test results close.

One option that fits well and is out of reach

There is one more thing you deserve to know about, even though nobody can act on it today. A treatment in early testing attaches a radioactive particle to a molecule that homes in on CLDN18.2. What makes it interesting for you specifically: the radiation kills not only the cell it lands on but its close neighbors too, so it matters much less that only 60% of your tumor cells carry the protein. On paper, this mechanism matches your tumor's staining pattern better than anything else Libby found, and your level clears the study's entry bar three times over.

The hard part is access. The only human studies are small, single-hospital trials in China, and studies of that kind have no mechanism for enrolling patients from abroad, however willing to travel you are. Right now there is no US or European program in this class at all. That can change, sometimes quickly. It is worth asking your team to keep a registry alert on this class, because a US trial opening would make this option real.

Questions to ask your oncologist

1. Can we schedule the biopsy of the new mass now, and can one procedure collect enough tissue for the CLDN18.2 re-stain, the genetic sequencing, and trial screening all at once?
2. Can we contact the daraxonrasib trial site and the expanded-access program before the biopsy result is back, so the six-week enrollment window does not close while we wait?
3. If you are considering starting chemotherapy first, would that dose make me ineligible for the daraxonrasib trial or any other trial on this list?
4. Can you order the dedicated germline genetic blood test this week, and formally grade the numbness in my hands and feet, since trials screen on both?
5. If the biopsy shows something other than recurrent pancreatic cancer, or shows the markers have changed, what is our plan then?
6. If the scans say the cancer is locally recurrent rather than spread, which options close, and can we have the ASP5834 trial screened as the backup before that answer arrives?
7. For the CLDN18.2 trials, will the sponsors accept my stored surgical tissue to start screening, and where does my staining level land against each trial's own cutoff?
8. If I start daraxonrasib, what is the plan for monitoring and managing rash and gut side effects, and what would we reach for when it eventually stops working?

Sources

The recommendations above draw on these published studies and trial registrations.

Publications (PubMed IDs): 23900716, 24277834, 31157963, 34433637, 35834777, 37068504, 37524953, 40670773, 40694660, 42080920, 42090791, 42223072. **Conference abstract (DOI):** 10.1200/JCO.2025.43.16_suppl.4017.

Trial registrations (ClinicalTrials.gov): NCT02628067, NCT03816163, NCT05365581, NCT05458219, NCT07094204, NCT07491445, NCT07573215, NCT07595237.